

Important Information. Please read carefully.

Xepasone[®]

Tablets

Composition

Each tablet contains Prednisolone 5 mg

Pharmacodynamics

Prednisolone is a synthetic glucocorticoid that is about four times more potent than hydrocortisone in glucocorticoid and anti-inflammatory activity, but with considerably less pronounced mineralocorticoid activity.

Pharmacokinetics

Prednisolone is readily absorbed via gastrointestinal tract. Peak plasma concentration is obtained 1 or 2 hours after oral dose; and has a usual plasma half-life of 2 to 4 hours. Prednisolone is extensively bound to plasma proteins. It is largely inactivated as it crosses the placenta and small amounts are excreted in breast milk. Most prednisolone are metabolised and excreted in the urine as conjugated metabolite and some excreted unchanged.

Indications

Suppression of inflammatory and allergic disorders including asthma, serum sickness, drug sensitisation, rheumatoid arthritis and urticaria.

Dosage and Administration

For oral use only.

Dosage requirements vary and must be adjusted according to individual response and the basis of the disease.

As a guide, the initial dosage range from 5 to 60 mg daily, in divided doses. After a favourable initial response is observed, the proper maintenance dosage should be determined by decreasing the initial dosage by small amounts to the lowest dosage that maintains an adequate clinical response. If the drug is to be stopped after more than a few days of treatment, it should be withdrawn gradually. In long-term therapy dosage should be maintained at not more than 7 mg daily whenever possible, as side effects occur with higher dosage.

Contraindications

Xepasone is similar to hydrocortisone and is contraindicated in patients with peptic ulcer, osteoporosis, psychoses or severe psychoneuroses, epilepsy, diabetes mellitus, tuberculosis, acute bacterial infection, herpes zoster, herpes simplex and other viral infections. Vaccination with live vaccine is contraindicated during therapy with **Xepasone**. It should be used with great caution in the presence of congestive heart failure, infectious diseases, chronic renal failure, uraemia and in elderly persons.

Warnings and Precautions

In patients subjected to unusual stress, increased dosage of rapidly acting corticosteroids before, during and after the stressful situation is indicated. Withdrawal of corticosteroids must be carried out gradually to avoid acute adrenal insufficiency. Aspirin should be used cautiously in conjunction with corticosteroids in hypoprothrombinemia. Steroids should be used with caution in non-specific ulcerative colitis, abscess, or other pyogenic infection, diverticulitis, active or latent peptic ulcer, renal insufficiency, hypertension and myasthenia gravis. When large doses are given, it is advisable to take with meals and antacids between meals to help prevent peptic ulcer.

Use in pregnancy should be avoided unless absolutely necessary and under close medical supervision. Avoid use in nursing mothers as corticosteroids may appear in breast milk. Patients on long term steroid therapy should be advised not to stop treatment except on medical advice and always have a supply in reserve. In case of feverish illness, accidents, operation (emergency or otherwise), diarrhoea or vomiting, the steroid treatment must be continued. Consultation with their physician is advisable for an increase dose or injection if necessary. Also seek medical advice if the tablets cause indigestion.

Patients should also inform any doctor, dentist, nurse or midwife, pharmacist or anyone else giving them treatment that they are on steroids or if after treatment, that they have been on steroids.

Scleroderma renal crisis

Caution is required in patients with systemic sclerosis because of an increased incidence of (possibly fatal) scleroderma renal crisis with hypertension and decreased urinary output observed with a daily dose of 15mg or more prednisolone. Blood pressure and renal function (s-creatinine) should therefore be routinely checked. When renal crisis is suspected, blood pressure should be carefully controlled.

Visual disturbance

Visual disturbance may be reported with systemic and topical corticosteroid use. If a patient presents with symptoms such as blurred vision or other visual disturbances, the patient should be considered for referral to an ophthalmologist for evaluation of possible causes which may include cataract, glaucoma or rare diseases such as central serous chorioretinopathy (CSCR) which have been reported after use of systemic and topical corticosteroids.

Drug Interactions

Concurrent use of barbiturates, phenytoin, or rifampicin may enhance the metabolism and decrease the effect of corticosteroids. If used with coumarin anticoagulants, the prothrombin time should be checked frequently; with potassium-depleting diuretics and carbenoxolone, patients should be observed closely for development of hypokalaemia. Antagonism may occur with antidiabetic drugs including insulin.

Co-treatment with CYP3A inhibitors, including cobistat-containing products, is expected to increase the risk of systemic side effects. The combination should be avoided unless the benefit outweighs the increased risk of systemic corticosteroid side-effects, in which case patients should be monitored for systemic corticosteroid side-effects.

Side Effects

Like other corticosteroids, prolonged use of **Xepasone** may exaggerate some of the normal physiological actions of the corticosteroids, resulting in the following adverse effects :

Fluid and electrolyte disturbances: Sodium and fluid retention, congestive heart failure in susceptible patients, potassium loss leading to hypokalemic alkalosis and hypertension.

Musculoskeletal: Muscle weakness, loss of muscle mass, osteoporosis and spontaneous fractures, tendon rupture and aseptic necrosis of bone.

Gastrointestinal: Peptic ulcer with possible perforation and haemorrhage, pancreatitis, abdominal distention, and ulcerative esophagitis.

Dermatologic: Impaired wound healing, thin fragile skin, petechiae and ecchymoses, erythema, increased sweating, possible suppression of reactions to skin tests, and other

cutaneous reactions such as allergic dermatitis, urticaria, angioneurotic oedema.

Neurologic: Convulsions, increased intracranial pressure, vertigo and headache.

Endocrine: Amenorrhoea, development of Cushing's syndrome, suppression of growth in children, secondary adrenocortical and pituitary unresponsiveness, particularly in times of stress, decreased carbohydrate tolerance, and manifestations of latent diabetes mellitus.

Ophthalmic: Posterior subcapsular cataracts, increased intraocular pressure, blurred vision and glaucoma.

Metabolic: Negative nitrogen balance due to protein catabolism.

Scleroderma renal crisis: Amongst the different subpopulations the occurrence of scleroderma renal crisis varies. The highest risk has been reported in patients with diffuse systemic sclerosis. The lowest risk has been reported in patients with limited systemic sclerosis (2%) and juvenile onset systemic sclerosis (1%).

Others: Hypersensitivity, thromboembolism, weight gain, increased appetite, nausea, malaise, and increased liability to infection.

Overdosage

Severe forms of adverse reactions may be observed in overdosage of **Xepasone**. A serious paranoid state or depression with risk of suicide may be induced, particularly in patients with a history of mental disorder. Euphoria is frequently observed. Signs of Cushing's syndrome, which include moonface, buffalo hump, flushing and bruising, striae and acne may be observed.

Treatment is symptomatic. The dosage should be reduced or the drug withdrawn gradually, to avoid acute adrenal insufficiency.

Shelf-life

The expiry date is indicated on the packaging.

Presentation

White, round tablets in dispensing pack of 1000's.

Storage

Store below 30°C.

KEEP OUT OF REACH OF CHILDREN / JAUHI DARI KANAK-KANAK

For further information, please consult your pharmacist or physician.

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